

PAD	CVI	DVT	Cellulitis
Patho: Arterial, lack of blood supply, tissues poorly perfused. Treat similarly to CAD (low fat diet, statins, etc)	Patho: Venous (aka PVD). Good blood supply, but weak veins/valves affect “venous return to the heart” Varicose veins inc. risk	Patho: blood clot in a deep vein in the legs – often hip to shin area. THINK: Virchow’s Triad & post-ortho Sx, sedentary (new/different) esp w/prolonged travel.	Patho: infection to deeper layers of skin usually from some type of break in the skin.
Dry skin, darker color, brown/dark red, wounds have “punched out” look to them. Less if any exudate, “crusty scab”, weak/absent pulses (may need to use doppler to assess) BILATERAL Increased risk for infection including Gangrene & difficult to Tx/heal infx	Wet & edematous, wounds are wet more weeping the worse it gets, pulses (may be hard to assess, push through pitting edema to feel or use doppler), Begin with BILATERAL edema that’s taught & shiny, legs begin to get darkened pink, after time darkened brownish/red color (staining),	Localized edema, red/darker red, warm, painful especially with movement, Don’t see wounds. D-Dimer elevation, Ultrasound. UNILATERAL	ANGRY RED (localized & often clearly demarcated), Warm/HOT , wounds definitely common, localized edema Infection/FLU LIKE SYMPTOMS <u>fever</u> , fatigue, increased WBCs , wound culture +, exudate serosanguinous and/or purulent pus , UNILATERAL ,
** Intermittent claudication Sharp pain when ambu. – rest & ambu. more. PROMOTE CIRCULATION – Gravity, LEGS DOWN!! NO COMPRESSION or ELEVATION!!!	PROMOTE VENOUS RETURN TO THE HEART – COMPRESSION socks, stocking, 🙌 ELEVATE!!!, plantar/dorsiflexion, diuretics	DEEP: Don’t massage/rub, ELEVATE, Ensure bedrest, Pharmacologic intervention. DON’T Plantar/dorsiflex (Homann’s sign – not anymore), bleeding risks due to anticoagulants	Risk for bone infection (osteomyelitis) & Sepsis. TX: Antibiotics , if wound – wound care.
A/BI - <0.9 indicative of PAD.		TX: IV HEPARIN – Oral anticoags (focus on prevention)	Wound cultures, blood cultures